

Suspected Child Abuse

- A high index of suspicion for child maltreatment must always be maintained.
- **It is a legal requirement to report all cases of suspected child abuse to the police (Child and Young Persons Act).**
- There may be many barriers to the maltreated child being recognised. Including:
 - The child may not recognise their experiences as abusive; perhaps because of normalisation within the household or cultural normalisation of abusive behaviour.
 - There may be feelings of shame, guilt or worries about stigmatisation.
 - They may be being coerced (or may be attached) to their abuser.
 - They may be fearful of the consequences of telling someone.
 - There are many barriers with an emergency department setting to the communication of their concerns, most notably lack of privacy.
 - Try to ameliorate as many of these barriers as possible.
- The following are the various types of abuse to be considered:
 1. **Physical abuse**
 2. **Emotional abuse**
 3. **Neglect or abandonment**
 4. **Sexual abuse**
 5. **Exploitation**
 6. **Confinement**

Diagnosis

Diagnosis requires a **high index of suspicion** and is based on a combination of medical findings that are unexplained, implausible or inconsistent with the history obtained, patterns of injury that suggest abuse and certain characteristics and behaviour of the child and family.

Risk factors for child abuse

<i>Abused Child</i>	<i>Abusive Parents</i>	<i>The family</i>
• Was unwanted • Poor bonding with parents • Is a disappointment, whether because of sex, a defect etc. • Is "irritable or demanding" • Is difficult to manage because of illness • Is different from the rest of the family	• Were abused or experienced family disruption in their childhood • Lack family support • Lack parenting skills and/or knowledge of child development, having unreasonable expectations • Have poor impulse	• Have employment and/or financial stress • Have marital conflict or domestic violence • Lack social supports • Experience crises due to stressful events such as a death in the family • History of child

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• Younger age (< 3 year olds are most likely to be physically abused)	control and are generally authoritarian • Young parents • Have physical or mental illness • Have problems with addiction	abuse in the family or poorly explained childhood deaths
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Clinical features that should raise the suspicion of abuse

- Vague, inconsistent, contradictory, inadequate or implausible story when explaining the cause of the child's injury.
- History provided, regarding the mechanism of injury, is not in keeping with the child's developmental abilities.
- Delay in seeking medical attention.
- Inappropriate parent or caregiver response.
- The child is not immunized.
- There is failure-to-thrive.
- A child with poor hygiene, dental and gum disease, or untreated sores.
- Sexual behaviour beyond the child's years and supposed knowledge.
- Multiple injuries of various types and of different ages.
- Fractures with high specificity for non-accidental injury (NAI) (see table below).
- Bruises or burns with particular patterns, e.g. 3 or 4 oval bruises suggestive of a slap on the face or a grasp around a limb; or burn marks characteristic of certain instruments such as cigarette burns or from irons.
- Bruises to padded areas like the buttocks, breasts, lower abdomen, or medial aspect of the thighs.
- The burn pattern of inflicted scalds may be seen in areas where accidental burns would not be expected, such as the lower limbs, buttock or perineal regions. Forced immersion would typically be a symmetric burn with sharply demarcated margins (often a glove and stocking distribution).
- Circular marks around the wrists or ankles suggestive of use of physical restraints.
- Genital injuries associated with a vague or inconsistent history.
- Head injuries with a vague or inconsistent history.
- Subdural haematoma associated with bilateral retinal haemorrhages in an infant, suggestive of **shaken baby syndrome**.
- Sexually transmitted disease in children.
- We need to exclude diseases which may present like NAI

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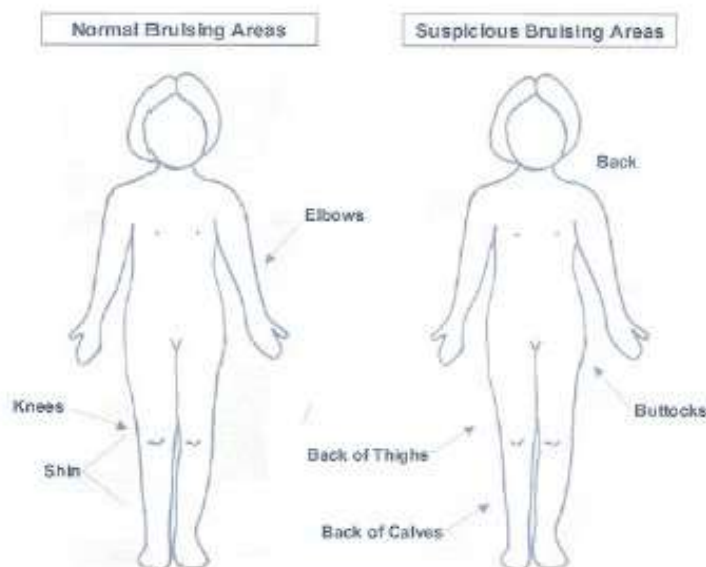
- All fractures/bruises could be sustained as a result of abuse but some as more commonly caused by this mechanism

Suspicious Fractures

Likelihood of non accidental trauma		
Low	Moderate	High
Clavicular fracture Long bone shaft fracture Linear skull fracture	Multiple Fractures (diff age) Complex Skull Fractures Vertebral body fracture/subluxation	Metaphyseal fracture Rib fracture Scapular fracture Sternal fracture

Skeletal trauma: general considerations. Kleinman PK et al
Diagnostic imaging of child abuse: Baltimore: Williams & Wilkins 1987;8-25

Normal and Suspicious Bruising Areas



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Differential diagnoses to consider

Presentation	Differential Diagnoses (not exhaustive)
Bruising	Coagulation Disorders Vasculitis Connective Tissue Disorder Metabolic- Scurvy/Menke's/ Glutaric Aciduria Type 1 Phyto dermatitis Mongolian spots Cultural practices- cupping Medications
Fractures	Osteogenesis Imperfecta Rickets Bony disease of prematurity Osteoporosis Insensitivity to pain disorders Accidental trauma- Toddler's fracture

Investigations:

Investigations are performed according to clinical judgment.

Common investigations (not necessarily required in the Children's Emergency at presentation):

- A skeletal survey is recommended particularly in young children (less than 24 months) with suspected physical abuse.
- A skeletal survey should also be considered for a child younger than 24 months who shares a home with an abused child.
- For children between the ages of 2-5 years, a skeletal survey MAY be indicated, especially if there is impaired mobility of the child (possibly due to injury) or impaired communication skills.
- It is uncommon for skeletal survey to be required for children with concerns for abuse who are above the age of 5 years.
- Tests for bleeding disorders can be considered in those who present with multiple non-specific bruises.
- For cases involving abandoned babies, all relevant medical screening, especially HIV testing should be carried out. HIV testing is authorized under the Protector's order or a Court Order.

Cases of suspected sexual abuse

For hot cases (<72 hours from alleged offence) the patient should be referred to:

Female: O&G registrar on call

Male: Paediatric Surgical Registrar

For cases beyond 72 hours, these referrals can be made as outpatients referrals or blue letter referrals in the ward.

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Please refer to the guidelines on alleged sexual assault for detailed information.

Photographic Evidence

The police have the primary responsibility of documenting photographic evidence of all injuries for the purpose of police investigation. As such, you should alert them early to any cases of suspected child abuse

No consent from the parents is necessary if the police take pictures. If the paediatrician-in-charge decides the pictures of injuries are necessary for clinical purposes, he/she should coordinate with the police to have the pictures taken in the same session, so as not to subject the CYP to additional inconvenience and trauma.

Documentation

Accurate documentation is essential! Good documentation is always important but particularly in the case of child abuse where there may be legal proceedings and thorough documentation of the given history and the physical examination would be vital. This should include:

- Standard, thorough history including medical and relevant social history
- Account given by the CYP, parent/caregiver and any significant other
- Location where the alleged abuse occurred
- Detailed description of the injuries: number, size, location
- An opinion on whether the injuries are consistent with the accounts given
- Results of all pertinent laboratory and other diagnostic procedures
- Any other significant facts or observations
- This will form the basis of subsequent medical reports

Disposition

All suspected cases of Child Abuse would be assessed in either the Children's Emergency or Children's Specialist Clinic. Decision to admit the child or otherwise will be dependent on:

- Medical needs
- Availability of safe care arrangement of the child. If a safe care arrangement (away from the alleged perpetrator) cannot be found, then the child should be admitted for protection.

Consultation may be made with the police and/or the MSF (Ministry of Social and Family Development) Officer on duty before arriving at a decision.

Outpatient management if:

- CYP has mild injuries that do not need inpatient management and
- The referring agency (police/ MSW/MSF) has a clear care and protection plan for the CYP and would follow up in monitoring the CYP's safety.
- The CYP can be housed away from the perpetrator and are deemed to be safe until further investigation is undertaken.

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Discharge planning consists of:

- Making a police report immediately (if no prior report was lodged)
- Referring the case to MSW (refer to workflow). The family will be seen on the next working day
- Clinic appointment to follow up and review medical conditions if necessary.

Admission is required for:

- All cases needing inpatient management of their injuries/medical condition
- There is no clear care and protection plan for the CYP on discharge
- Discharging the CYP may compromise his/her safety

Admission planning consists of:

- Making a police report immediately (if no prior report was lodged)
- CYP should be admitted under Paediatric Medicine discipline who will then coordinate with other specialists if required
- Referral to the Medical Social Worker within 1 working day (by the inpatient managing team)
- The Senior Doctor on shift should be consulted if there is difficulty in managing the CYP and the family. It is important to maintain a calm and non-confrontational attitude. The medical staff must remain polite but firm, and be honest with the parents about the concerns raised regarding the CYP's injuries and the CYP's future safety and welfare. Focus on the CYP's welfare and safety.
- Medical staff can seek police assistance to authorize the admission of the CYP in the hospital, which is gazetted as a place of safety, under Section 9(1) of the CYPA. This is applicable if the initial assessment suggests the CYP's injuries are severe; His safety is compromised or parents are uncooperative and insist on discharging the CYP against medical advice. If the parents abscond with the child, the police should be informed immediately.

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